

# Sacral Neuromodulation (Bladder and Bowel Pacemaker)

WARWIKI

Patient Information · An implanted device that steadies bladder and bowel control

Sacral neuromodulation (SNM) uses a small implanted device, sometimes called a “bladder pacemaker.” It gently stimulates the **sacral nerves** near the tailbone, which help control the bladder and bowel. It is used for overactive bladder, some types of trouble emptying the bladder, and accidental bowel leakage. A key strength is that you **test it first**, before the permanent implant.

## How It Works

The sacral nerves carry signals between the bladder, the bowel and the brain. SNM sends **mild electrical pulses** that help steady those signals. This can calm urgency and leaks, or help a bladder that does not empty. You can adjust or pause it with a handheld remote.

## What It Treats

- **Overactive bladder** (urgency, frequent trips, urge leaks)
- **Urinary retention** not caused by a blockage (a bladder that will not empty)
- **Accidental bowel leakage** (fecal incontinence)

## Benefits and Risks in Numbers

about  
**7 in 10**

with urge leakage were still helped 5 years after the implant

about  
**1 in 3**

needed another procedure on the device within 3–4 years

about  
**4 in 100**

got an infection at the implant site in the first year

Numbers are averages from published studies. Your own risk may differ; ask your team.

## Learn the Terms

### Sacral nerves

Nerves near the tailbone that help control the bladder and bowel.

### Neuromodulation

Using gentle electrical pulses to steady nerve signals.

### Test phase

A trial with a thin wire to see if it helps before the permanent implant.

### Generator

The small battery device placed under the skin of the upper buttock.

### Lead

The thin wire that carries the pulses to the nerve.

### Remote

A handheld controller to adjust or pause the device.

### MRI

A scan that uses a strong magnet. Some devices limit which MRI scans you can have.

**How do I know if it will work for me?** You **try it first**. A thin wire is placed and you track your symptoms in a diary for a few days to a few weeks. You move on to the permanent implant only if your symptoms improve by about half or more.

#### Test phase

Keep a diary. Avoid bending, twisting and hard activity so the wire does not move.

#### After the implant

Some soreness for a few days. Your team sets up the device.

#### Over the years

Setting changes, battery care, and sometimes another procedure.

## How It's Done (Two Steps)

- 1 Test phase:** a thin wire is placed near a sacral nerve, in the office or an operating room. You get numbing medicine, sometimes with medicine to relax you. The wire connects to a small device worn outside the body.
- 2 Implant:** if the test works, a small **generator** is placed under the skin of the upper buttock and joined to the wire. You usually go home the same day.
- 3** Your team programs the device. You use a **remote** to adjust it.

#### Call the office that did your procedure if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- Redness, swelling or fluid at the wound
- New or worse pain at the device site
- A sudden loss of benefit, or stimulation that feels uncomfortable

#### Go to an emergency department if:

- You cannot urinate for several hours and your bladder feels full or painful
- You cannot reach the office and are worried

## Living With the Device

- Some batteries are recharged at home. Others last many years and are then replaced.
- **Before any MRI**, show your device card. Many newer systems allow MRI scans under set conditions. Some older wires allow head scans only.
- Never have **diathermy** (deep heat therapy).
- If you become pregnant, tell your team. The device is usually turned off.

#### Three Things to Remember

- 1. You test it first.** The permanent implant is placed only if it clearly helps.
- 2. It can be adjusted, turned off or removed.**
- 3. Show your device card before any MRI.**